

PROGRAM OVERVIEW

Faculty Orientation



Methodist

HEALTHCARE

Graduate Medical Education

Bravein Amalakuhan, MD

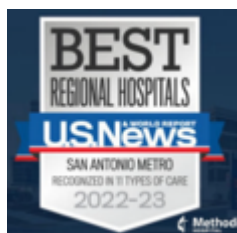
Program Director, IM Residency

Methodist Healthcare; San Antonio, TX

Methodist Healthcare System

Exceptional Healthcare Accessible to All

The mission of Methodist Healthcare is 'Serving Humanity to Honor God' by providing exceptional and cost-effective health care accessible to all'



- America's 50 Best Surgical Care (2024, 2023)
- America's 100 Gastrointestinal Surgery (2024, 2023)
- America's Best Cardiac Care (2024, 2023)
- America's 100 Orthopedic Surgery (2024, 2023)
- America's 100 Best Stroke Care (2024, 2023)
- America's 100 Prostate Surgery (2024, 2023)
- America's 100 Coronary Intervention (2024, 2023, 2022)
- America's 100 Pulmonary Care (2024)
- America's 100 Critical Care (2024, 2023, 2022)
- Neurosciences Excellence Award (2024)
- America's 100 Gastrointestinal Care (2024, 2023)
- Vascular Surgery Excellence Award (2024, 2023)



10 Acute Care Hospitals

1 under development
4 managed rural hospitals

10 Freestanding ERs

3 under development

6 Ambulatory Surgery Centers

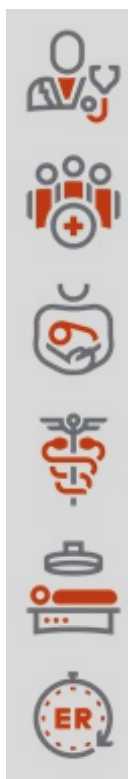
21 Urgent Care Centers

1 under development

50+ Physician Clinic Locations

26 Imaging Centers (STRIC JV)

2 under development



3.47k Medical Staff

12.5k+ Colleagues

11.9k+ Deliveries

111k+ Admissions

67.7k+ Surgeries

357k+ ER Visits

Internal Data: FY 2022

WE TRANSFORM
LIVES

Elements of a Successful GME Program



GROWTH & INNOVATION

THE FUTURE



WHY GME? – Enhancing the Mission of MHS “Accessible to All”

~60% of Residents End Up Practicing In The Same City They Trained In

SAN ANTONIO

- Physician Deficits Expected by 2024
 - ~142 IM Physicians
 - ~100 Family Practice Physicians (PCPs)
 - ~40 General Surgeons

Nationally

- >150,000 deficit of Physicians by 2024
- 35% of the Physician workforce will be of retirement age within 5-years

WHY GME? – Improving Quality Outcomes



Original Investigation | Medical Education

Association of Graduate Medical Education With Hospital Performance and Patient Outcomes

Radoslav Zinoviev, MD, MBA; Harlan M. Krumholz, MD, SM; Kevin Pirruccio, BA; Howard Forman, MD, MBA

Research Report

Quality of Care in Teaching Hospitals: A Literature Review

Joel Kupersmith, MD

REVIEW ARTICLE

OPEN ACCESS [Check for updates](#)

Benefits of developing graduate medical education programs in community health systems

Richard Alweis , Anthony Donato , Richard Terry , Christina Goodermote , Farrah Qadri^a and Robert Mayo^a

^aDepartment of Medical Education, Rochester Regional Health, Rochester, New York, United States; ^bDepartment of Medicine, Tower Health, West Reading, Pennsylvania, United States; ^cAcademic Affairs, Lake Erie College of Medicine at Elmira, Elmira, New York, United States

The Role of Graduate Medical Education (GME) in Improving Patient Safety

Arpana R. Vidyarthi, MD; Robert B. Baron, MD, MS | February 1, 2010

JAMA[®]

The Journal of the American Medical Association



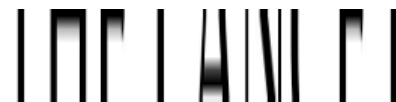
National Institutes of Health
Clinical Center

ACADEMIC MEDICINE

Journal of the Association of American Medical Colleges



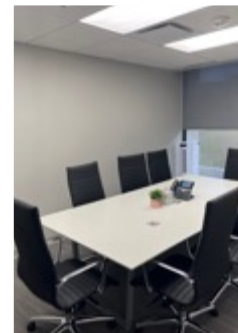
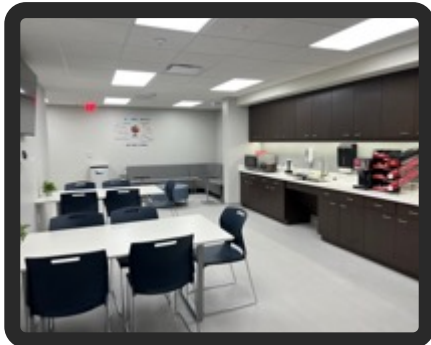
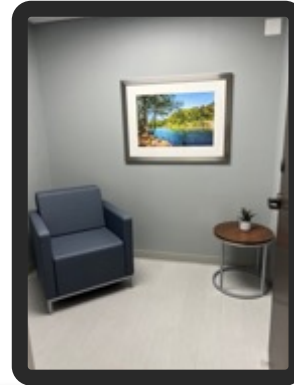
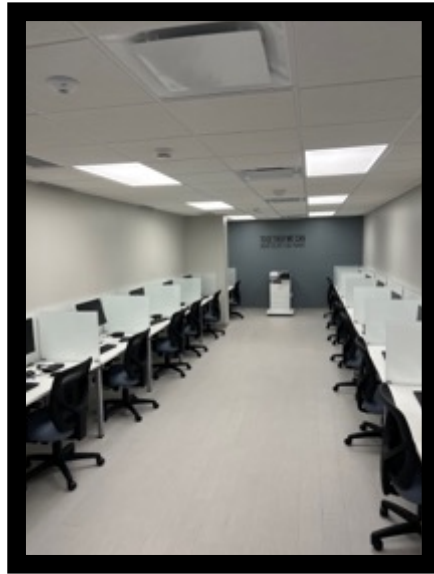
The NEW ENGLAND
JOURNAL of MEDICINE



CMS

CENTERS FOR MEDICARE & MEDICAID SERVICES

WHERE WE ARE

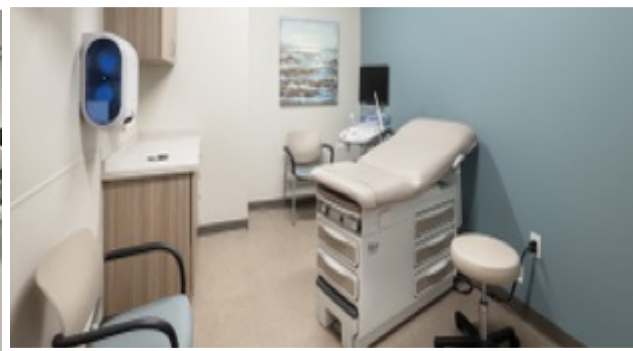
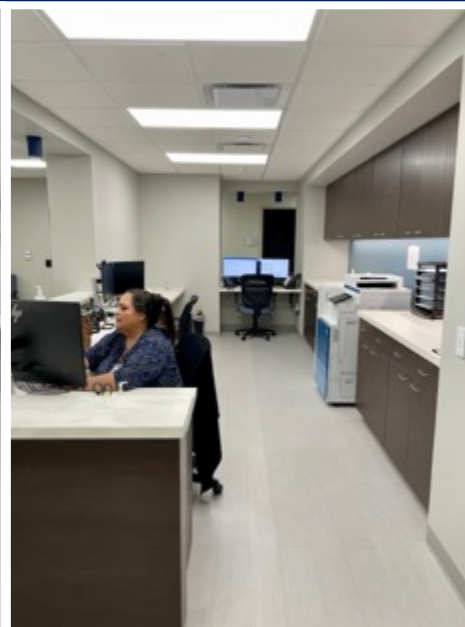
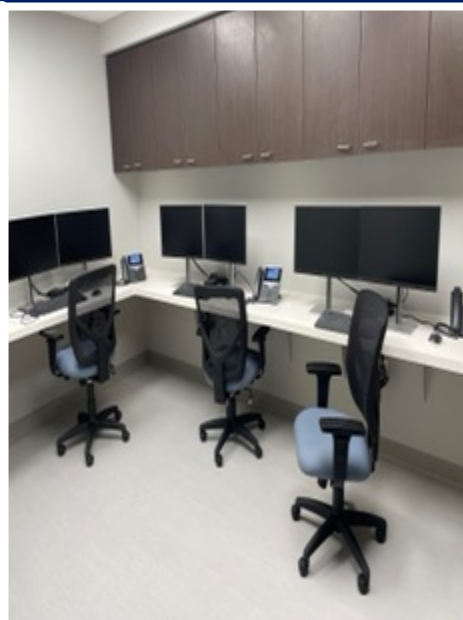


- *Skylighted seating in front of GME Offices*
- *Dedicated Resident Workroom*
- *Large Breakroom (microwave, 2 fridges)*
- *Quiet rooms*
- *Administrative Conference Room*
- *Large Debrief room (not pictured)*
- *Call rooms in hospitals (not pictured)*
- *Lockers with Keypads (not pictured)*
- *Program leadership offices (not pictured)*
- *Lactation Room (not pictured)*
- *Physician Gym available (not pictured)*
- *Simulation Center (OR, Procedural Skill Rooms, Sim Console Room etc)*

THANK YOU METHODIST HEALTHCARE !

GME Suite – 7900 SQUARE FEET
(1st Floor Methodist Plaza)

WHERE WE ARE



Continuity Clinic (2nd Floor Methodist Plaza)

HOW?: PROGRAM STRUCTURE

MHS GME Policies / R & R



Methodist Healthcare System of San Antonio

Graduate Medical
Education Manual



Graduate Medical
Education Committee
July 2023

AVAILABLE TO ALL FACULTY

HOW?: PROGRAM STRUCTURE

IM Curriculum – Block Schedule (4 + 1 model)



PGY1	CCIM	MICU	InptMed	NF	EM	Neph	ID	Pulm	Cards	Electives
	10wks	8wks	12wks	4wks	4wks	4wks	4wks	4wks	4wks	4wks
PGY2	CCIM	MICU	InptMed	NF	Rheum	Neuro	GI	HEM/ONC	Electives	
	10wks	8wks	12wks	4wks	4wks	4wks	4wks	4wks	8wks	
PGY3	CCIM	MICU	InptMed	NF	H & P	Endo	Add/GERI	Electives		
	10wks	8wks	12wks	4wks	4wks	4wks	4wks	12wks		

****Rotation schedule is specific to each resident per PGY level.**

LEGEND

CCIM = Continuity Clinic – Internal Medicine
MICU = Medical Intensive Care Unit
InptMed = Inpatient Medicine
NF = Night Float
EM = Emergency Medicine

Neph = Nephrology
ID = Infectious Disease
Pulm = Pulmonology
Cards = Cardiology
Rheum = Emergency Medicine

Neuro = Neurology
GI = Gastroenterology
HEM/ONC = Hematology / Oncology
H & P = Hospice & Palliative Care
Endo = Emergency Medicine
ADD/GERI = Addiction & Geriatrics

Participating Sites:

- Methodist Hospital
- Methodist Specialty & Transplant Hospital
- Methodist Hospital – Metropolitan

EVIDENCE BASED CURRICULUM

HOW?: PROGRAM STRUCTURE

IM Curriculum – Didactics

ORIENTATION

- *Onboarding (now until June)*
- *Orientation Week (Last week of June 2024)*
- *Intern Boot Camp (June/July)*

MONTHLY

- *Journal Club*
- *Board Review*
- *M & M Conference*
- *Tumor Board*
- *Scholarly Activity / Research*
- *Grand Rounds*
- *Well-Being Sessions*

DAILY

- *Morning Report / Case Conferences*
- *Faculty Teaching Rounds*

ACADEMIC HALF-DAY (Weekly - Thursday's 12pm – 5pm)

- *Allows for focus on didactic training away from patient care responsibilities (MHS service lines are amply covered by faculty / attendings – not resident dependent)*
- **SIMULATION TRAINING**
- *ACLS Training*
- **POC-US CURRICULUM**
- *Resident lead sessions as program matures*
- *Frequent breaks in between sessions to avoid fatigue and information overload*

SUPERVISION STRUCTURE

Inpatient Medicine / MICU Rotations

PGY1s:

MUST not be assigned more than **5** new patients per admitting day

MUST not be assigned **8** new patients in a 48hr period

MUST not be responsible for the ongoing care of more than **10** patients

WHEN NOT SUPERVISING

PGY2s & PGY3s:

MUST not be responsible for the ongoing care of more than **10** patients

WHEN SUPERVISING:

PGY2s & PGY3s

When **supervising** more than one PGY1 resident, the PGY2 or PGY3 MUST not be responsible for the supervision or admission of more than **10** new patients and **4** transfers per admitting day

When **supervising** more than one PGY1 resident, the PGY2 or PGY3 MUST not be responsible for more than **16** new patients in a 48hr period

PGY2 or PGY3 MUST not be responsible for the ongoing care of more than **14** patients when supervising one PGY1 and not more than **20** patients when supervising more than one PGY1.

PGY1s = 3

(Intern)



PGY1s = 2

(Intern)

PGY2 or PGY 3 = 1

(mid-level senior)



PGY1s = 1

(Intern)

PGY2s = 1

(mid-level senior)

PGY3s = 1

(senior)



Inpatient Medicine Rotation

- Monday to Sunday
- Hours: 6 am -6 pm
- Structured Hand-Offs

Morning Report

- M, T, W, F
- Hours 7:30 am -8:30 am

Faculty: In-house 24/7 Supervision

ACCOUNTABILITY STRUCTURE

Graduate Medical Education Committee (GMEC)

**Governing body within MHS that
maintains authority / responsibility
over each ACGME Accredited
Program**

Clinical Competency Committee (CCC)

Appointed by Program Director
Reviews Resident Evaluations Quarterly
Evaluates Progress on Specific Milestones
Can meet Ad Hoc
Advises PD on Resident Progress / Deficiencies



Elements of a Successful GME Program

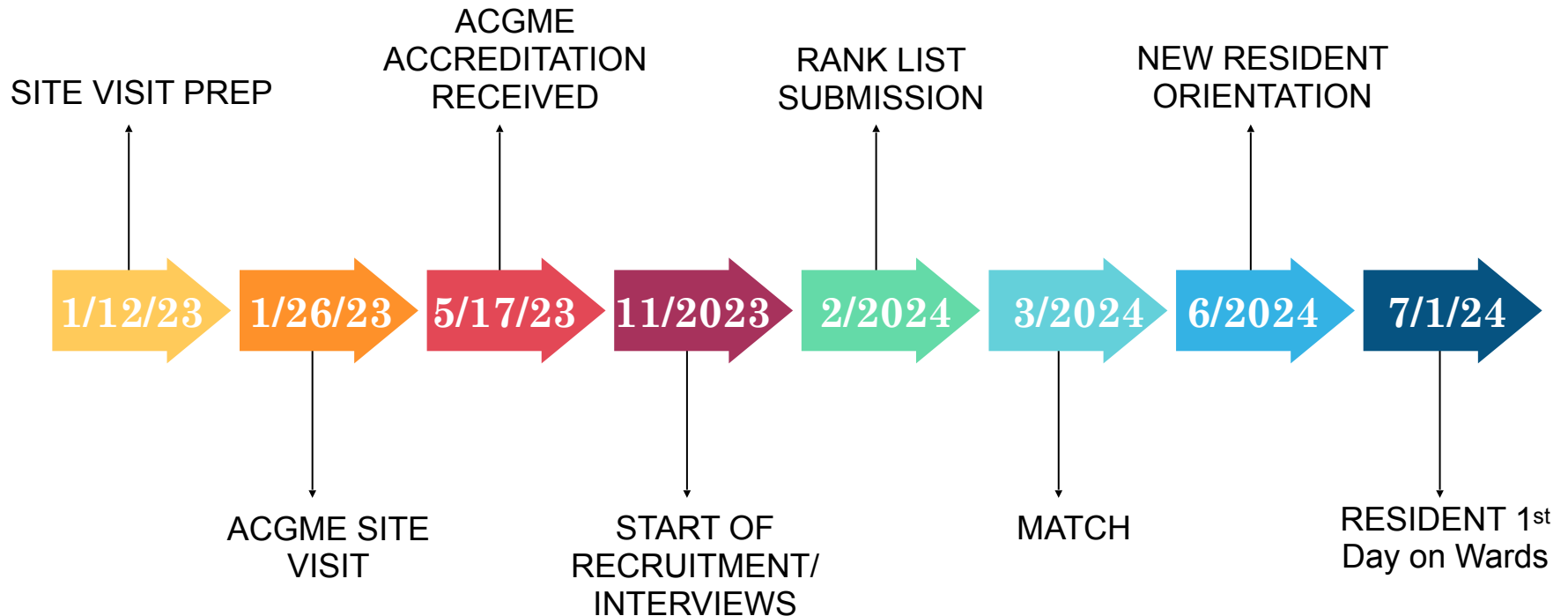


APPENDIX

- GME Journey Timeline
- GME Leadership Structure
- GME Faculty List and Rotations
- Description of Levels of Supervision
- Handoffs & Transitions of Care Policy

HOW WE GOT HERE

GME JOURNEY: TIMELINE



WHO WE ARE

GME Organization Chart – MHS SA Overall

Designated Institutional Official (DIO)
Graduate Medical Education Committee
(GMEC) Chair
Justin Williams, MD

Institutional Director, GME
Bianca Rodriguez, MHA

Program Director – Internal Medicine
Bravein Amalakuhan, MD

Program Administrator
Internal Medicine
Maggie Medrano & Myriam Gandy

Associate Program Directors *

CORE Faculty*

Teaching Faculty*

Residents

Program Director – Surgery
Travis Holloway, MD

Program Administrator
Surgery
Elaine Zamarripa

Associate Program Directors *

CORE Faculty*

Teaching Faculty*

Residents

Program Director – OB/GYN
Elly Xenakis, MD

Program Administrator
OB/GYN
Krystal Trevino

Associate Program Directors *

CORE Faculty*

Teaching Faculty*

Residents

Program Director – Podiatry
Thomas Zgonis, DPM

Program Administrator
Podiatry
Myriam Gandy

Associate Program Directors *

CORE Faculty*

Teaching Faculty*

Residents

PROGRAM LEADERSHIP: IM

Program Director
Bravein Amalakuhan, MD

Associate Program Director
Director of Education
Jairo Melo, MD

Associate Program Director
Director of Strategic Growth
Fernando Triana, MD

Associate Program Director
Director of Clinical Operations
Shankar Sanka, MD

Subspecialty Director of Pulmonology
Director of Patient Safety
Ali Abedi, MD

Teaching Faculty
Director of Recruitment & Marketing
Nandish Thukral, MD

Core Faculty
Director of Scheduling & System Integration
Neil Patel, MD

Subspecialty Director of Critical Care
Director of Simulation & AI
Hitesh Gidwani, MD

Subspecialty Director of Cardiology
Director of Innovation
Ananth Prasad, MD

Director of Outpatient Medicine
Ambassador to the ACP
Claudia Paz, MD

Teaching Faculty
Director of Point-of-Care-US (POCUS)
Fernando Vasquez, MD

Core Faculty
Director of Wellness, Diversity & Inclusion
Faraz Fiazuddin, MD

Core Faculty
Director of Hospitalist Medicine
Bishal Sinha, MD

Core Faculty
Director Preventative Medicine
Marichel Marcolino, MD

Core Faculty
Director of Quality Improvement
Vamsi Surapaneni, MD

Core Faculty
Director of Community Outreach
Gopi Chandrahasan, MD

Research Faculty: Director – Dr.Kari Nilsen

Research Coordinator – Clarissa Villarreal

- Dr. DellaVolpe
- Dr. Burdick
- Anthony Shamoun
- Linda Sousse
- Dr. Burch
- Dr. Abedi
- Dr. Jorge Alvarez
- Dr. Donovan
- Dr. D Patel
- Dr. Iturbe
- Dr. D Pederson
- Dr. Adeola
- Dr. T Triana
- Dr. Mauro Ortiz
- Dr. A Baum
- Dr. N Thukral
- Dr. A Martinez
- Dr. K Nguyen

WHO WE ARE



Program Director
Dr. Bravein Amalakuhan



Associate Program Director
Dr. Fernando Triana



Associate Program Director
Dr. Jairo Melo



Associate Program Director
Dr. Shankar Sanka



Program Administrator
Maggie Medrano



Methodist
HEALTHCARE
Graduate Medical Education

Categorical Internal Medicine Residency

Resident Levels of Supervision

DIRECT

- *the supervising physician is physically present with the resident during the key portions of the patient interaction*

INDIRECT

- *the supervising physician is not providing physical or concurrent visual or audio supervision but is immediately available to the resident for guidance and to provide appropriate direct supervision*

OVERSIGHT

- *the supervising physician is available to provide review of procedures/encounters with feedback provided after care is delivered.*

PROGRESSIVE AUTHORITY

- *Recognized as a Privilege*
- *Delegated by the Program Director and Faculty*
- *Delegated based on needs of the patient and skills of the resident*
- *Senior residents can serve in supervisory role to junior residents*

HANDOFFS & TRANSITIONS OF CARE POLICY

HANDOFFS AND TRANSITIONS OF CARE

HANDOFFS AND TRANSITION OF CARE POLICY

I. Purpose:

To establish protocol and standards within the MBIS Office of Graduate Medical Education residency and fellowship programs to ensure the quality and safety of patient care when transfer of responsibility occurs during work hour shift changes and other scheduled or unscheduled circumstances. To assure continuity of care and patient safety, the Accreditation Council for Graduate Medical Education (ACGME) requires a minimum number of patient care transitions, a structured and monitored handoff process, training for competency by residents in handoffs, and readily available schedules listing residents and attending physicians responsible for each patient's care. In addition to resident-to-resident patient transitions, residents must care for patients in an environment that maximizes effective communication among all individuals or teams with responsibility for patient care in the healthcare setting.

II. Definition:

A handoff is defined as the communication of information to support the transfer of care and responsibility for a patient group of patients from one provider to another. Transitions of care are necessary in the hospital setting for various reasons. The transition/hand-off process is an interactive communication process of passing specific, essential patient information from one caregiver to another. Transition of care occurs regularly under the following conditions:

1. Change in level of patient care, including inpatient admission from an outpatient procedure or diagnostic area or ER and transfer to or from a critical care unit.
2. Temporary transfer of care to other healthcare professionals within procedure or diagnostic areas.
3. Discharge, including discharge to home or another facility such as skilled nursing care.
4. Change in provider or service change, including change of shift for nurses, resident hand-off, and rotation changes for residents.

III. Policy:

Each training program must design schedules and clinical assignments to maximize the learning experience for residents as well as to ensure quality care and patient safety and adhere to general institutional policies concerning transitions of patient care within the context of other clinical and educational work hour standards. All training programs must design call and shift schedules to optimize transitions in patient care, including their safety, frequency, and structure. Schedule overlaps should include time to allow for face-to-face handoffs to ensure availability of information and an opportunity to clarify issues. Programs must have in place specific processes for handover of patient care in all relevant scenarios (night/weekend coverage, rotating off-service, vacation coverage, between services and locations (OR – ICU) etc.).

HANDOFFS AND TRANSITIONS OF CARE

IV. Procedure:

A. The transition/hand-off process should involve face-to-face interaction with both verbal and written/computerized communication, with opportunity for the receiver of the information to ask questions or clarify specific issues. Hand-offs can be conducted over the phone as long as both parties have access to an electronic or hard copy version of the sign-out sheet. Additionally, all attempts to preserve patient confidentiality are observed. The transition process should include, at a minimum, the following information in a standardized format that is universal across all services:

1. Identification of patient, including name, medical record number, and date of birth.
2. Name and contact number of responsible resident and attending physician.
3. Diagnosis and current status/condition (level of acuity) of patient to include resuscitation status.
4. Recent events, including changes in condition or treatment, current medication status, recent lab tests, allergies, anticipated procedures, and actions to be taken.
5. Outstanding tasks – what needs to be completed in immediate future.
6. Outstanding laboratory studies – what needs follow up during shift.
7. Changes in patient condition that may occur requiring interventions or contingency plans i.e., situational awareness.
8. Include synthesis of information by the provider being briefed, such as "read-back" or asking questions to confirm understanding.

B. Each residency program must develop components ancillary to the institutional transition of care policy that integrate specifics from their specialty field. Programs are required to develop scheduling and transition/hand-off procedures to ensure that:

1. Residents comply with specialty specific/institutional duty hour requirements.
2. Faculty are scheduled and available for appropriate supervision levels according to the requirements for the scheduled residents.
3. All parties (including nursing) involved in a particular program and/or transition process have access to one another's schedules and contact information. All call schedules should be available on department-specific password-protected websites and also with the hospital operators.
4. Patients are not inconvenienced or endangered in any way by frequent transitions in their care.
5. All parties directly involved in the patient's care before, during, and after the transition have opportunity for communication, consultation, and clarification of information.
6. Each program must ensure continuity of patient care, consistent with the program's policies and procedures in the event that a resident may be unable to perform their patient care responsibilities due to excessive fatigue or illness, or family emergency.
7. Programs should provide an opportunity for residents to both give and receive feedback from each other or faculty physicians about their handoff skills.

C. Each program must include the transition of care process in its curriculum.

HANDOFFS AND TRANSITIONS OF CARE

D. Residents must demonstrate competency in performance of this task. There are numerous mechanisms through which a program might elect to determine the competency of trainees in handoff skills and communication. These include:

1. Direct observation of a handoff session by a supervising faculty, chief resident, or supervising resident.
2. Evaluation of written handoff materials by supervising faculty, chief resident, or supervising resident.
3. Didactic sessions on communication skills including in-person lectures, web-based training, review of curricular materials and/or knowledge assessment.
4. Assessment of handoff quality in terms of ability to predict overnight events.
5. Assessment of adverse events and relationship to sign-out quality through:
 - a) Survey
 - b) Reporting hotline
 - c) Chart review

E. Programs must develop and utilize a method of monitoring the transition of care process and update as necessary. Monitoring of handoffs by the program to ensure:

1. There is a structured, standardized process in place that is routinely followed such as IPASS (attachment A).
2. There is consistent opportunity for questions.
3. The necessary materials are available to support the handoff (including, for instance, written sign-out materials, access to electronic clinical information).
4. A quiet setting free of interruptions is consistently available for handoff processes that include face-to-face communication.
5. Patient confidentiality and privacy are ensured in accordance with HIPAA guidelines.
6. Programs must include a standardized question on the end of rotation/assignment faculty evaluation of resident, i.e. "Did you or a supervising resident/fellow observe this resident during a handover process? If yes, is this resident competent in communicating in the handover process?"
7. Handover evaluation template may be used by programs if so desired (Attachment B).

F. GME/C ensures and monitors effective structured handover processes to facilitate both continuity of care and patient safety through annual review of program specific handover policies and ACGME resident and faculty survey results.

Attachment A

The IPASS mnemonic can be used as a way to standardize the verbal (oral) hand-off process at periods of shift change during in-person verbal communication. It can also be used as a framework to standardize the written handoff process by integrating the individual mnemonic elements in computerized handoff tools within word processing documents or, ideally, within the electronic medical record where possible.

Mnemonic Letter	Description	Key Points
I	Illness Severity	• Identification of patient's level of acuity to focus attention appropriately at the start of the handoff communication • Suggest classifying each patient using a standardized language such as stable, "watcher" (a patient whose any clinician has a concern that a patient is at risk of deterioration), or unstable • May include code status • Classification may vary depending on unit acuity, provider type, or institutional culture
P	Patient Summary	• Describe succinctly the reason for admission, events leading up to admission, hospital course, and plan for hospitalization • Should reflect global plan for entire hospital stay and avoid "to-do" items for next shift • Should be maintained and updated regularly with modification of assessment, diagnosis, and changes in treatment plans as necessary • Events leading up to admission may be truncated with time and diagnosis certainty but should retain key reason for admission to allow new providers to understand reasons of presentation
A	Action Items	• Includes a "to-do" list with specific elements to accomplish over next shift by team assuming care of patient • Should specify timeframe for completion, level of priority, and who is responsible • Specify "nothing to do" if no action items are anticipated
S	Situational Awareness and Contingency Plans	• Situational Awareness: knowing what is going on for members of the care team (status of patients, environmental factors, team members) and for each individual patient (status of disease process, progress towards goals for hospitalization) • Contingency Plans: with situational awareness in mind, provide team assuming care of the patient with specific instructions for how to handle anticipated problems • Typically includes "if/then" statements • Specify "no contingencies anticipated" for stable patients • Ensure accepting team is prepared to anticipate changes in patient status and respond to potential events
S	Synthesis by Receiver	• Provide a brief re-statement of essential information in a urgent summary to receiving team • Demonstrate information is received and understood • Ensure effective transfer of information and responsibility • Opportunity for receiver to clarify elements of handoff, ensure clear understanding, and play an active role in handoff process • Will vary in length and content depending on acuity level of patient